

PROCESS FOR GA MRI FOR NEUROCRITICAL CARE PATIENTS

Request from parent team for GA MRI for neurocritical care patient.

NCC doctor to determine scan required (MRI brain, spine, MRA, vessel wall imaging, perfusion and spectroscopy) as well as urgency of MRI from parent team – can the case be deferred until after extubation

ELECTIVE

Review of patients requiring GA MRI after Monday Neurology MDT and Tuesday NCC MDT to determine clinically urgency. Final list to be decided at end of Tuesday MDT meeting with priority if more than one case

Long/extended day doctor responsible looking after the patient, to order MRI to be protocolised. NCC team to inform patient's family of MRI and document in notes.

Extended day GFICU doctor to email the agreed GA MRI patients for Thursday afternoon list **before 17:00 on Tuesday**. Email to include details patient's name, DOB, MRN, bed space number as well as MRI type requested. A summary of patient's clinical history, ventilatory, CVS and any other organ support. Highlight any surgical implants such as coils, EVD, etc. Email thread to include NCC consultants for the week as well as consultant anaesthetist for the list (from CLWrota)

CLW Rota Consultant lucia.misquita1@nhs.net

MRI Senior radiographers lisa.strycharczuk@nhs.net ; rebecca.hill30@nhs.net

Angio Senior radiographers david.mcguinness3@nhs.net ; dominic.holleron@nhs.net

Lead ODP and theatre admin coordinator liam.purcell@nhs.net ; m.dee@nhs.net

If there are any changes to the clinical condition of the patient or change in the urgency of the MRI scan, please let the above email thread recipients know.

URGENT

Parent team to request MRI to be protocolised and inform patient's family of MRI

NCC team to add patient to GA MRI on emergency pink list and inform 8011 SpR

NCC consultant to discuss case with anaesthetic consultant floor co-ordinator before the 07:45 emergency theatre team brief, to allow planning of the emergency list for that day.

NCC doctors to liaise with MRI radiographers for time slots to allow GA MRI on that day.

NCC consultant and anaesthetic floor coordinator consultant to liaise during the day to facilitate the MRI scan. Possible solutions should be discussed and take into account the number of patients on NCC, availability of NCC consultant or senior NCC trainee, any available anaesthetic consultants where lists may have been cancelled.

If patient's MRI rolls over to the next day, NCC consultant to rediscuss with parent team on urgency, as well as anaesthetic floor coordinator as to options to facilitate scan.